



SURGICAL PATHOLOGY REPORT

Patient Name: [REDACTED]
Med. Rec. #: [REDACTED] Visit #: [REDACTED]
DOB: [REDACTED] Sex: Male
Soc. Sec. #: [REDACTED] Location: [REDACTED]
Physician(s): [REDACTED]

Accession #: [REDACTED]
Service Date: [REDACTED]
Received: [REDACTED]
Client: [REDACTED]

ICD O-3
Carcinoma, urothelial NOS 8120/3
Site Bladder, lateral wall
CGZ 2
7/28/14

FINAL PATHOLOGIC DIAGNOSIS

- A. Left ureter, biopsy: Ureter with no significant pathologic abnormality; no tumor identified.
- B. Right ureter, biopsy: Ureter with no significant pathologic abnormality; no tumor identified.
- C. Bladder and prostate, radical cystoprostatectomy:
1. Invasive high-grade urothelial carcinoma, 3.0 cm, invading perivesicular fat, negative margins; see comment.
 2. Extensive carcinoma in situ extending to bladder dome.
 3. Prostate with no significant pathologic abnormality, no prostatic or urothelial carcinoma identified.
 4. Bilateral seminal vesicles with no significant pathologic abnormality.
 5. Bilateral ureters with no significant pathologic abnormality.
- D. Bladder, "more," excision: Fibrofatty tissue; no tumor identified.
- E. Right pelvic lymph nodes, dissection: No tumor identified in thirteen lymph nodes (0/13).
- F. Left pelvic lymph nodes, dissection: No tumor identified in eight lymph nodes (0/8); see comment.
- G. Meckel's diverticulum, excision: Small bowel mucosa with focal gastric heterotopia; no tumor identified.
- H. Left ureter, "final," excision: Ureter with no significant pathologic abnormality.

I. Right ureter, "final," excision: Ureter with no significant pathologic abnormality.

COMMENT:

The urothelial carcinoma does not invade the prostate. No prostatic adenocarcinoma is identified, and no high grade prostatic intraepithelial neoplasia is seen.

An immunohistochemical stain for keratin was performed on slide F1, and is negative, ruling against metastatic carcinoma.

Bladder Tumor Synoptic Comment

- **Tumor type:** Urothelial (transitional cell) carcinoma.
- **Tumor grade:** High grade.
- **Tumor size:** 3.0 cm.
- **Extent of tumor in bladder:** Invasion through muscularis propria into perivesical tissue.
- **Lymphatic/vascular invasion:** None.
- **Epithelial abnormalities in bladder:** Carcinoma in situ.
- **Extension of tumor into adjacent organs:** None.
- **Margins:** Negative (invasive tumor is 0.3 cm from margin).
 - **Right ureter margin:** Negative.
 - **Left ureter margin:** Negative.
 - **Perivesical margin:** Negative (invasive tumor is 0.3 cm from margin).
- **Lymph node status:** Negative.
- **Total number of nodes examined:** 21.
- **Total number of positive nodes:** 0.

- **Other pathologic findings in bladder:** None.
- **AJCC/UICC stage:** pT3aN0MX.

Specimen(s) Received

A:Left ureter, biopsy (FS)
B:Right ureter, biopsy (FS)
C:Bladder (FS)
D:More bladder specimen (fresh)
E:Right pelvic lymph node
F:Left pelvic lymph node
G:Meckel's "diverticle"
H:Final left ureter
I:Final right ureter

Intraoperative Diagnosis

FS1 (A) Left ureter, biopsy: No carcinoma. (Dr.

FS2 (B) Right ureter, biopsy: No carcinoma. (Dr.

FS3 (C) Urethral margin, biopsy: No carcinoma. (Dr.

Clinical History

The patient is a [redacted] year-old man with urothelial carcinoma who now undergoes radical cystoprostatectomy.

Gross Description

The specimen is received in nine parts, each labeled with the patient's name and medical record number. Parts A and B are received fresh for frozen section diagnosis. Parts D-I are received in

formalin.

Part A, additionally labeled "1 - left ureter," consists of one soft, irregular, unoriented, white-pink-red, irregular tissue fragment that is 0.7 x 0.5 x 0.2 cm. The entire specimen is frozen for frozen section diagnosis 1, and subsequently submitted in cassette A1.

Part B, additionally labeled "2 - right ureter," consists of one soft, irregular, unoriented, white-pink-red, irregular tissue fragment that is 1.2 x 0.6 x 0.3 cm. The entire specimen is frozen for frozen section diagnosis 2, and subsequently submitted in cassette B1.

Part C is received fresh additionally labeled "bladder," and consists of a 288-gram bladder and prostate measuring 13.5 x 11 x 4.5 cm. The urethral margin is submitted for frozen section diagnosis #3. The bladder mucosa shows a 3 x 2 cm, pink-tan, papillary mass that is on the left wall and extends from the trigone to the left dome. The tumor extends into the muscularis propria 2cm, and appears to abut but not involve the serosal surface. The mass does not appear to involve the perivesicular fat. The mass is 2 cm from the prostate. The remaining bladder mucosa is tan-pink, folded and unremarkable. The attached prostate measures 4 x 3 x 3 cm, and on cut section, contains white-tan, whorled, grossly unremarkable prostate parenchyma. The attached seminal vesicles each measure 3 x 2 x 2 cm appear grossly unremarkable. The perivesicular fat is removed, and lymph nodes are thoroughly searched for, however, and no lymph nodes are found. Cassettes are submitted as follows:

Cassette C1: Frozen section 3 frozen section remnant.
Cassettes C2-C5: Full thickness sections showing deepest extent of invasion.
Cassette C6: Relationship of mass to adjacent bladder mucosa.
Cassettes C7-C8: Consecutive sections showing relationship of the mass to the prostate.
Cassette C9: Right bladder dome.
Cassette C10: Right bladder wall.
Cassette C11: Right inferior bladder wall.
Cassettes C12-C13: Prostate apex after removal of the urethral margin.
Cassettes C14-C15: Mid-portions of the prostate.
Cassette C16: Left prostate base with seminal vesicles.
Cassette C17: Right prostate base with seminal vesicles.

Part D, labeled "#4 - more bladder specimen," consists of a single soft, irregular piece of fat measuring 3 x 2.5 x 0.3 cm. One portion of this fatty tissue has a stitch attached, and this stitch may be a retraction aid. Nonetheless, the area is inked black. The stitch is removed. This stitched area has a fibrous, beefy appearance and consistency. The specimen is entirely submitted with the firm fibrous tissue in cassette D1 and the remaining fatty tissue in cassette D2.

Part E, additionally labeled "5 - right pelvic LN," consists of multiple tan-brown-yellow fatty tissue fragments measuring 5.5 x 4 x 1.5 cm in aggregate. The specimen is trimmed and extensively searched for lymph nodes. Multiple candidate lymph nodes are found and the entire specimen is submitted as follows:

Cassette E1: Four intact nodes.
Cassette E2: Largest intact node.
Cassettes E3-E4: Remainder of specimen.

Part F, additionally labeled "6 - left pelvic LN," consists of multiple soft, tan-yellow, fatty tissue fragments measuring 5 x 4 x 1.5 cm in aggregate. The specimen is trimmed and extensively searched for lymph nodes. Multiple candidate lymph nodes are found and the entire specimen is submitted as follows:

Cassette F1: Four intact nodes.
Cassette F2: Three intact nodes.
Cassette F3: Remaining fat

Part G, additionally labeled "7 - Meckel's diverticle [sic]," consists of one firm, pink-tan, unoriented tissue fragment measuring 1.7 x 0.8 x 0.8 cm. On one end, the specimen reveals a lumen that measures 0.2 cm in diameter with a pink-tan ring. The specimen is inked blue, then longitudinally transected reveal soft and firm, pink-tan homogeneous tissue. No lesions are noted. The entire

[REDACTED]
specimen is submitted in cassette G1. [REDACTED]

Part H, additionally labeled "8 - final left ureter," consists of one soft, tan-light brown tissue fragment oriented by a black suture and metal staple on one end, measuring 1 x 0.7 x 0.4 cm. The suture and the staple line are removed, and the underlying area is inked blue. The specimen is cross-sectioned and entirely submitted in cassette H1.

Part I, additionally labeled "9 - final right ureter," consists of one soft, tan-brown tissue fragment oriented by a tan suture and a metal staple on one end measuring 1.1 x 0.8 x 0.4 cm. The suture and the staple are removed and the underlying area is inked blue. The specimen is cross-sectioned and entirely submitted in cassette I1. ([REDACTED])

The immunoperoxidase stain(s) reported above were developed and their performance characteristics determined by the [REDACTED]. They have not been cleared or approved by the U. S. Food and Drug Administration. The FDA has determined that such clearance or approval is not necessary. These tests are used for clinical purposes. They should not be regarded as investigational or for research. This laboratory is certified under the Clinical Laboratory Improvement Amendments of 1988 ("CLIA") as qualified to perform high-complexity clinical testing.

Diagnosis based on gross and microscopic examinations. Final diagnosis made by attending pathologist following review of all pathology slides.

[REDACTED] Pathology Resident

[REDACTED] /Pathologist
Electronically signed out on [REDACTED]

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary		☒
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	[Signature]	
Date Reviewed	10/24/2013	